

Abnormal/Clinical Psychology

*Comprehensive EPPP Preparation Study Guide
For Destyne | Dr. DeDe's Concept List*

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1. Major Depressive Disorder (5 of 9 Criteria)

DSM-5-TR Criteria: Five or more of the following during same 2-week period (must include #1 or #2):

1. Depressed mood most of the day, nearly every day 2. Diminished interest or pleasure in activities (anhedonia) 3. Significant weight change or appetite disturbance 4. Insomnia or hypersomnia 5. Psychomotor agitation or retardation 6. Fatigue or loss of energy 7. Feelings of worthlessness or excessive guilt 8. Diminished ability to think or concentrate 9. Recurrent thoughts of death or suicidal ideation

Specifiers (8): With anxious distress, with mixed features, with melancholic features, with atypical features, with psychotic features, with catatonia, with peripartum onset, with seasonal pattern.

Source: DSM-5-TR (2022)

2. Manic Symptoms (3 of 7 Criteria)

Manic Episode - DSM-5-TR: Distinct period of abnormally elevated/expansive/irritable mood lasting ≥ 1 week. Three or more of the following (four if mood is only irritable):

1. Inflated self-esteem or grandiosity 2. Decreased need for sleep 3. More talkative than usual or pressure to keep talking 4. Flight of ideas or subjective experience that thoughts are racing 5. Distractibility 6. Increase in goal-directed activity or psychomotor agitation 7. Excessive involvement in activities with high potential for painful consequences

Source: DSM-5-TR (2022)

3. Bipolar I Disorder

DSM-5-TR Criteria:

Manic Episode Required: At least one lifetime manic episode (duration ≥ 1 week, or any duration if hospitalized).

Depressive Episodes: Usually present but not required for diagnosis.

Manic Episode Criteria: Elevated/expansive/irritable mood + 3+ symptoms (or 4+ if irritable only) causing marked impairment or hospitalization.

Duration: Mania ≥ 7 days (or hospitalization); Hypomania ≥ 4 days.

Mixed Features Specifier: Can be applied when features of opposite pole present during episode.

Source: DSM-5-TR (2022)

4. Bipolar II Disorder

DSM-5-TR Criteria:

1. At least one **hypomanic episode** (≥ 4 days, less severe than mania, no hospitalization/psychosis, noticeable but not severe impairment)

2. At least one **major depressive episode**

3. Never had a full manic episode

Key Difference from BP I: Presence of hypomania (not mania) + depression; no history of full mania.

Cyclothymic Disorder: Numerous periods of hypomanic symptoms and depressive symptoms (not meeting full criteria) for ≥ 2 years.

Source: DSM-5-TR (2022)

5. Manic Episode Length & Duration Criteria

Type	Duration	Severity	Hospitalization
Mania	≥ 7 days	Severe	May be hospitalized
Hypomania	≥ 4 days	Less severe	No hospitalization

Note: Mania can be diagnosed if hospitalized regardless of duration.

Source: DSM-5-TR (2022)

6. Mixed State of Bipolar Disorder

Definition: Episode with simultaneous symptoms of opposite polarity (manic + depressive symptoms at the same time).

How Common: Occurs in approximately 20-40% of individuals with bipolar disorder.

Clinical Significance: Associated with worse outcomes, increased suicide risk, more frequent episodes, greater treatment resistance.

DSM-5-TR: Mixed features specifier can be applied to manic, hypomanic, or depressive episodes.

Source: DSM-5-TR (2022); Goldberg, J. F., & McElroy, S. L. (2019). Mixed features in bipolar disorder. *CNS Spectrums*.

7. Conduct Disorder - 15 Symptoms

DSM-5-TR 15 Symptoms (3+ in past 12 months, 1+ in past 6 months):

Aggression to People/Animals: 1. Bullies, threatens, intimidates 2. Initiates physical fights 3. Used weapon causing serious harm 4. Physically cruel to people 5. Physically cruel to animals 6. Stolen while confronting victim 7. Forced sexual activity

Destruction of Property: 8. Deliberate fire-setting 9. Deliberately destroyed property

Deceitfulness or Theft: 10. Broken into building/house/car 11. Lies to obtain goods/favors 12. Stolen items of nontrivial value without confrontation

Serious Violations of Rules: 13. Out late at night before age 13 14. Run away from home overnight ≥ 2 times 15. Truant from school before age 13

Subtypes: Childhood-onset, Adolescent-onset, Unspecified onset.

Source: DSM-5-TR (2022)

8. Conduct Disorder - Three Subtypes

DSM-5-TR Subtypes:

1. **Childhood-Onset Type:** At least one criterion prior to age 10 - More likely to develop Antisocial Personality Disorder - More severe symptoms, greater impairment
2. **Adolescent-Onset Type:** No criteria prior to age 10 - Less severe, better prognosis - Less likely to persist into adulthood
3. **Unspecified Onset:** Age of onset unknown

Severity Specifiers: Mild, Moderate, Severe (number of conduct problems and harm to others).

Source: DSM-5-TR (2022)

9. Oppositional Defiant Disorder (ODD)

DSM-5-TR Criteria: Pattern of angry/irritable mood, argumentative/defiant behavior, or vindictiveness lasting ≥ 6 months.

Four Symptom Categories: 1. Angry/Irritable mood 2. Argumentative/Defiant behavior 3. Vindictiveness (spiteful/vindictive ≥ 2 times in 6 months)

ODD vs CD Key Differences: - ODD: Less severe, no violation of others' rights, no physical aggression to people/animals, no destruction of property, no theft/deceit - CD: More severe, violates rights of others, physical aggression, property destruction, theft - ODD may develop into CD

Source: DSM-5-TR (2022)

10. Disruptive Mood Dysregulation Disorder (DMDD)

DSM-5-TR Four Main Criteria:

1. **Severe recurrent temper outbursts** (verbal/behavioral) grossly out of proportion to provocation
2. **Outbursts inconsistent with developmental level**
3. **Outbursts occur 3+ times per week** (on average)
4. **Persistently irritable/angry mood** between outbursts, present most of the day, nearly every day

Additional Requirements: - Onset before age 10 - Duration ≥ 12 months - Never without symptoms for > 3 months - Present in ≥ 2 settings - Not diagnosed before age 6 or after age 18 - Never meets criteria for mania/hypomania > 1 day

Source: DSM-5-TR (2022)

11. ASD Diagnostic Criteria (2 Main)

DSM-5-TR Autism Spectrum Disorder - Two Domains:

A. Persistent Deficits in Social Communication and Interaction: - Deficits in social-emotional reciprocity - Deficits in nonverbal communicative behaviors - Deficits in developing/maintaining relationships

B. Restricted, Repetitive Patterns of Behavior, Interests, or Activities: - Stereotyped/repetitive motor movements, speech, or use of objects - Insistence on sameness, inflexible routines - Highly restricted, fixated interests - Hyper- or hyporeactivity to sensory input

Requirements: Symptoms present in early development; cause significant impairment; not better explained by intellectual disability.

Specifiers: With/without accompanying intellectual impairment, language impairment, associated with known medical/genetic/environmental factor, associated with another neurodevelopmental/mental/behavioral disorder.

Source: DSM-5-TR (2022)

12. ASD - Three Common Comorbid Conditions

Three Most Common:

1. **Intellectual Disability** (~50% of ASD cases; though many have average/above-average intelligence)
2. **ADHD** (high comorbidity; impulsivity, inattention)
3. **Anxiety Disorders** (especially social anxiety, specific phobias, generalized anxiety)

Also Common: - Depression - Epilepsy/seizure disorders - Sleep disturbances - Gastrointestinal problems - Feeding/eating issues - Obsessive-compulsive disorder

Source: DSM-5-TR (2022); Simonoff, E., et al. (2008). Psychiatric disorders in children with autism spectrum disorders. *Journal of the American Academy of Child and Adolescent Psychiatry*.

13. Substance Use Disorders (10 Major Criteria)

DSM-5-TR Substance Use Disorder Criteria (2+ in 12 months):

1. Taking substance in larger amounts or longer than intended
2. Persistent desire or unsuccessful efforts to cut down
3. Great deal of time spent obtaining, using, or recovering
4. Craving or strong desire to use
5. Failure to fulfill major role obligations
6. Continued use despite social/interpersonal problems
7. Important activities given up or reduced
8. Recurrent use in physically hazardous situations
9. Continued use despite physical/psychological problems
10. Tolerance (need for increased amount or diminished effect)
11. Withdrawal (characteristic syndrome or use to relieve/avoid withdrawal)

Severity: Mild (2-3), Moderate (4-5), Severe (6+)

Source: DSM-5-TR (2022)

14. Intellectual Disability (3 Key Diagnostic Criteria)

DSM-5-TR Criteria for Intellectual Developmental Disorder:

1. **Deficits in Intellectual Functioning** - Reasoning, problem-solving, planning, abstract thinking, judgment - IQ score approximately 70 or below (clinical + standardized testing)
2. **Deficits in Adaptive Functioning** - Failure to meet developmental/sociocultural standards - Limits functioning in activities of daily life
3. **Onset During Developmental Period** - Must manifest before age 18 - Cannot be diagnosed if deficits begin after developmental period

Severity Levels: - Mild (IQ 50-55 to 70) - Moderate (IQ 35-40 to 50-55) - Severe (IQ 20-25 to 35-40) - Profound (IQ below 20-25)

Source: DSM-5-TR (2022)

15. Learned Helplessness Model of Depression

Definition: Depression results from attributing negative events to internal, stable, global causes, leading to passivity and hopelessness.

Whose Model: Martin Seligman (reformulated by Abramson, Seligman, & Teasdale, 1978)

Attributional Dimensions: - **Internal vs. External:** Is the cause me or the situation? - **Stable vs. Unstable:** Will this cause persist? - **Global vs. Specific:** Does this affect all areas of life?

Depressive Attributional Style: Internal, Stable, Global attributions for negative events.

Source: Seligman, M. E. P. (1975). *Helplessness: On depression, development, and death*. Freeman.

16. Depressive Disorder Specifiers (8)

DSM-5-TR Specifiers for Depressive Disorders:

1. **With anxious distress:** Presence of anxiety symptoms 2. **With mixed features:** Some manic/hypomanic symptoms present 3. **With melancholic features:** Anhedonia, non-reactive mood, early morning awakening 4. **With atypical features:** Mood reactivity, weight gain, hypersomnia, leaden paralysis, sensitivity to rejection 5. **With psychotic features:** Delusions or hallucinations (mood-congruent or incongruent) 6. **With catatonia:** Stupor, rigidity, excessive motor activity 7. **With peripartum onset:** Onset during pregnancy or within 4 weeks postpartum 8. **With seasonal pattern:** Regular temporal relationship to particular time of year

Source: DSM-5-TR (2022)

17. ADHD Diagnostic Criteria

DSM-5-TR ADHD Criteria: Persistent pattern of inattention and/or hyperactivity-impulsivity interfering with functioning.

Inattention (6+ symptoms for children, 5+ for adults): - Careless mistakes, difficulty sustaining attention, doesn't listen, doesn't follow through, difficulty organizing, avoids sustained effort, loses things, easily distracted, forgetful

Hyperactivity-Impulsivity (6+ for children, 5+ for adults): - Fidgets, leaves seat, runs/climbs inappropriately, can't play quietly, "on the go," talks excessively, blurts out, difficulty waiting turn, interrupts

Presentation Specifiers: - Combined presentation - Predominantly inattentive presentation - Predominantly hyperactive-impulsive presentation

Source: DSM-5-TR (2022)

18. Anxiety Disorders Overview

DSM-5-TR Anxiety Disorders:

Separation Anxiety Disorder: Developmentally inappropriate fear of separation from attachment figures.

Selective Mutism: Consistent failure to speak in specific social situations despite speaking in other situations.

Specific Phobia: Marked fear/anxiety about specific object/situation.

Social Anxiety Disorder: Fear of social situations where one may be scrutinized.

Panic Disorder: Recurrent unexpected panic attacks + persistent concern/worry.

Agoraphobia: Fear/anxiety of situations where escape might be difficult.

Generalized Anxiety Disorder: Excessive anxiety/worry occurring more days than not for ≥ 6 months.

Source: DSM-5-TR (2022)

19. Schizophrenia Diagnostic Criteria

DSM-5-TR Criteria (2+ symptoms for ≥ 1 month):

Characteristic Symptoms (at least 1 must be 1, 2, or 3): 1. Delusions 2. Hallucinations 3. Disorganized speech 4. Grossly disorganized/catatonic behavior 5. Negative symptoms (diminished emotional expression/avolition)

Duration: Continuous signs ≥ 6 months (including prodromal/residual phases)

Functional Impairment: In work, interpersonal relations, or self-care.

Source: DSM-5-TR (2022)

20. Personality Disorders Clusters

DSM-5-TR Personality Disorder Clusters:

Cluster A (Odd/Eccentric): - Paranoid: Distrust/suspiciousness - Schizoid: Detachment from relationships, restricted affect - Schizotypal: Acute discomfort in relationships, cognitive/perceptual distortions, eccentricities

Cluster B (Dramatic/Emotional/Erratic): - Antisocial: Disregard for/violation of rights of others - Borderline: Instability in relationships, self-image, affect; impulsivity - Histrionic: Excessive emotionality/attention seeking - Narcissistic: Grandiosity, need for admiration, lack of empathy

Cluster C (Anxious/Fearful): - Avoidant: Social inhibition, feelings of inadequacy, hypersensitivity to negative evaluation - Dependent: Submissive/clinging behavior, need to be taken care of - Obsessive-Compulsive: Preoccupation with orderliness, perfectionism, control

Source: DSM-5-TR (2022)

21. Trauma and Stressor-Related Disorders

DSM-5-TR Categories:

Reactive Attachment Disorder: Consistent pattern of emotionally withdrawn behavior toward caregivers; limited positive affect; episodes of unexplained irritability/sadness/fearfulness.

Disinhibited Social Engagement Disorder: Pattern of overly familiar behavior with unfamiliar adults; reduced reticence; willingness to go off with strangers.

PTSD: Exposure to actual/threatened death, serious injury, or sexual violence; intrusion symptoms; avoidance; negative alterations in cognition/mood; alterations in arousal/reactivity.

Acute Stress Disorder: Similar to PTSD but duration 3 days to 1 month after trauma exposure.

Adjustment Disorders: Emotional/behavioral symptoms in response to identifiable stressor within 3 months of onset.

Source: DSM-5-TR (2022)

22. Eating Disorders

Anorexia Nervosa: - Restriction of energy intake leading to significantly low body weight - Intense fear of gaining weight/becoming fat - Disturbance in experience of body weight/shape

Bulimia Nervosa: - Recurrent episodes of binge eating - Recurrent inappropriate compensatory behaviors (vomiting, laxatives, fasting, excessive exercise) - Self-evaluation unduly influenced by body shape/weight

Binge Eating Disorder: - Recurrent episodes of binge eating - No regular inappropriate compensatory behaviors - Associated with eating rapidly, until uncomfortably full, when not hungry, alone, feeling disgusted/depressed/guilty

Source: DSM-5-TR (2022)